IRBESARTAN

Once daily
Teratogenic

CHEMICAL NAME 2-butyl-3-({4-[2-(2H-tetrazol-5-yl)phenyl]phenyl}methyl)-1,3-diazaspiro[4.4]non-1-en-4-one

CLASS Angiotensin II receptor blocker (ARB)

TRADE NAMES Aprovel · Avapro · CoAprovel (with HCTZ) · Irban

THERAPEUTIC USES / INDICATIONS

- Hypertension.
- Type 2 diabetic nephropathy with hypertension (slows progression).

PHARMACODYNAMICS

- Selective **AT1 receptor** antagonist → vasodilation, ↓ aldosterone.
- Reduces proteinuria and slows diabetic kidney disease.

ADMINISTRATION

- **PO** once daily with or without food.

DOSAGES

- **Hypertension:** 150 mg once daily; may increase to 300 mg.
- Start 75 mg in elderly >75 yr, haemodialysis or volume depletion.
- **Diabetic nephropathy:** 300 mg once daily.

PHARMACOKINETICS

- **Onset:** 1–2 h; maximal effect 4–6 weeks. **Duration:** 24 h.
- Bioavailability 60–80%; metabolised in liver (CYP2C9, glucuronidation).
- Excreted in bile and urine. **t½:** 11–15 h.

SIDE & ADVERSE EFFECTS

SIDE EFFECTS (COMMON)

- Dizziness, fatigue.
- Musculoskeletal pain.
- Nausea.

ADVERSE EFFECTS (SERIOUS)

- Hyperkalaemia.
- Renal impairment.
- Orthostatic hypotension.
- Angioedema (rare).

CONTRAINDICATIONS & CAUTIONS

- Pregnancy.
- Aliskiren in diabetes or eGFR <60.
- **Caution:** renal artery stenosis, volume depletion.

KEY INTERACTIONS

- **K^+-sparing diuretics:** hyperkalaemia.
- **NSAIDs:** AKI.
- **Lithium:** toxicity.

NURSING MANAGEMENT

- Monitor BP, K^+, creatinine and urine protein.
- Assess hydration.
- Postural BP in elderly.
- Educate on diabetic kidney care.

PATIENT EDUCATION

- Take daily even when well.
- Report dizziness or fainting.
- Avoid potassium supplements and NSAIDs.
- Avoid pregnancy.

KEY POINTS

- Once-daily ARB.
- Hyperkalaemia.
- Diabetic nephropathy (300 mg).
- No pregnancy.